**New Somerset Hospital**
Department of Anaesthesia

PATIENT STICKER

Name .....
Folder no. .....
Date of birth .....

# REGIONAL ANAESTHESIA CONSENT — Truncal & Fascial Plane Block

## 1. PLANNED PROCEDURE & BLOCK

Procedure ..... Side: Left Right Bilateral

Block(s): ☐ Paravertebral ☐ Erector spinae (ESP) ☐ Intercostal ☐ Serratus anterior ☐ PECS I / II
☐ Parasternal ☐ TAP ☐ Rectus sheath ☐ Quadratus lumborum ☐ Ilioinguinal / IH
Other .....

Planned with: ☐ Block alone ☐ Sedation ☐ GA ☐ Spinal ☐ Site marked

Patient has capacity to consent: Yes No If no, consent given by (name / relationship) .....

## 2. BENEFITS DISCUSSED

*why a block is recommended for this patient*

- ☐ Excellent pain relief at the operation site (often 8–24 h)
- ☐ Less opioid needed: less nausea, vomiting, drowsiness, itch, constipation
- ☐ Rib fractures / chest surgery: deep breathing, coughing & physio easier → fewer chest infections
- ☐ Alternative to epidural (e.g. on anticoagulants) with fewer side effects: hypotension, urinary retention, leg weakness
- ☐ Earlier eating, drinking, mobilising and discharge

Other benefit .....

## 3. RISKS DISCUSSED

*most patients have no complications · details and figures in the patient leaflet*

| | | |
|---|---|---|
| **General — all nerve and plane blocks** | | |
| ☐ | **Common** (1 in 10–100) | Prolonged numbness / weakness (expected 8–24 h) · Partial or failed block · Rebound pain as the block wears off · Bruising / tenderness · Temporary tingling or numbness |
| ☐ | **Uncommon** (1 in 100–1 000) | Bleeding / haematoma |
| ☐ | **Rare** (1 in 1 000–10 000) | Local anaesthetic toxicity (fits, heart rhythm) · Permanent nerve damage (> 6 months) |
| ☐ | **Very rare** (< 1 in 10 000) | Infection · Severe allergic reaction |
| **Block-specific** | | |
| ☐ | **Common** (1 in 10–100) | Visceral pain not covered · Hypotension / epidural spread |
| ☐ | **Uncommon** (1 in 100–1 000) | Pneumothorax (collapsed lung) · Leg weakness / numbness |
| ☐ | **Rare** (1 in 1 000–10 000) | LA toxicity with bilateral blocks |
| ☐ | **Very rare** (< 1 in 10 000) | Bowel or organ puncture |
| ☐ | Other risk discussed ..... | |

## 4. ALTERNATIVES DISCUSSED

- ☐ Oral / IV analgesia (incl. opioids or PCA)
- ☐ Local anaesthetic infiltration by surgeon
- ☐ General anaesthetic without a block
- ☐ A different block

## 5. AFTERCARE ADVISED

- ☐ Regular analgesia before the block wears off
- ☐ Report breathlessness or chest pain (pneumothorax)
- ☐ Assisted first mobilisation
- ☐ Report numbness / weakness lasting > 48 h

## 6. CONFIRMATION

Right to refuse explained (no effect on surgery or care): Yes No ☐ Questions answered

Patient leaflet: QR shown Printed copy Declined

Interpreter: Not needed Used Language ..... Name .....

**Patient gives consent for the block(s) above:** YES NO

Patient leaflet

Doctor ..... MP no. ..... Signature .....

Date ..... Time .....

Patient leaflet: omicronblade.github.io/Anaes-Doc-Repo/anaesthesia-forms/leaflets/regional-truncal.html



SASA/SASRA Guidelines 2016 · McCombe & Bogod, Anaesthesia 2021 · frequencies approximate